

Workplace Violence Prevention

Staff Safety, De-escalation, and Incident Response

Regulation: 42 CFR §482.13(e) | OSHA
General Duty Clause

POLICY OVERVIEW

Healthcare workers face disproportionately high rates of workplace violence. Hospitals have a legal and ethical obligation to provide a safe work environment. A comprehensive Workplace Violence Prevention Program (WVPP) must address prevention, de-escalation, response, reporting, and recovery.

KEY REQUIREMENTS

1 Workplace Violence Risk Assessment

A systematic risk assessment of all hospital areas must be conducted to identify environmental, administrative, and behavioral risk factors for violence. High-risk areas (ED, psychiatric units, waiting areas) require enhanced preventive measures. Risk assessments must be updated annually and after incidents.

2 Prevention Strategies

Prevention measures must include: visitor management and access control, duress alarm systems in high-risk areas, metal detection in appropriate settings, patient behavioral risk flagging systems, adequate staffing levels (inadequate staffing increases violence risk), and environmental design modifications.

3 De-escalation Training

All clinical staff must receive training in verbal de-escalation techniques before patient care assignment and annually thereafter. Training must include: recognition of escalating behavior, communication strategies, safe positioning, and when and how to request assistance.

4 Incident Reporting and Response

All workplace violence incidents must be reported without fear of retaliation. A standardized reporting process must be in place. After any violent incident: ensure staff safety, provide medical care, initiate the incident reporting process, preserve evidence, and notify appropriate parties including security and administration.

5 Post-Incident Support

Staff involved in violent incidents must have access to: employee assistance program (EAP), peer support, management debriefing, and time off if needed. Critical incident stress debriefing should be offered after serious incidents. Return-to-work support must be provided.

COMPLIANCE NOTES

OSHA's Guidelines for Preventing Workplace Violence for Healthcare and Social Service Workers provides a recommended framework. Several states have enacted specific workplace violence prevention laws for healthcare. Leadership commitment is the single most important predictor of program effectiveness.

COMMON SURVEY CITATIONS

Supported under §482.13(e) – Safe environment for staff, and OSHA General Duty Clause requiring employers to provide a workplace free from recognized serious hazards.